

eCBHFA Facilitator guide

Module: Maternal and Newborn Child Health

Topic: Breastfeeding

Overview

Advice and support for women to breastfeed; advising women who are living with HIV about breastfeeding.

At the end of this topic, volunteers should be able to:

- say whether most women in your community breastfeed their babies
- say why breastfeeding is important and good for the baby
- explain when a mother should begin breastfeeding her newborn baby and how long a mother should breastfeed her infant
- say how best to support mothers to breastfeed and how to help the family to support them
- advise what a mother living with HIV should do about breastfeeding
- teach advice and support about breastfeeding content to their community

Topic tools

- Benefits of breastfeeding poster
- How to breastfeed poster
- Common problems with breastfeeding poster

Suggested outline of activities

Depending on the number of volunteers and amount of discussion, this topic will take approximately 1 hour, 55 minutes to complete.

Activity	Time
Introduction	15 min
Acceptance of breastfeeding in the community	40 min
Breastfeeding support	20 min
Reflection	10 min
Helping mothers learn to breastfeed and common problems with breastfeeding	20 min
Wrap-up	10 min
Total time	1 hr 55 min

Preparation

- Write the learning objectives for the topic on flipchart paper and post at the front of the room.
- Prepare copies of the topic and tools for each trainee.

Topic summary

Knowledge about the common breastfeeding practice in the community is an important first step for volunteers. Volunteers should know how important it is that they encourage new mothers to breastfeed exclusively for the first six months. They can help mothers to learn to breastfeed and can advise those women who have problems such as soreness. They can also give advice to mothers living with HIV about breastfeeding.

Introduction

1. Welcome the volunteers to the topic. Tell them that this topic examines breastfeeding in much more detail than the previous topic. Stress how important a topic it is. Breastfeeding is a practice that they should encourage throughout the community.
2. Introduce the topic objectives and explain the agenda and activities that they will do to meet those objectives.
3. Distribute the topic **Breastfeeding** from the volunteer guide.
 - a. Explain that volunteers will be taught how to use three tools during the session and that they will be expected to practise using these tools and demonstrate their skill in delivering the content.
 - b. Allow approximately two minutes for the volunteers to become familiar with the topic materials.
4. Instruct volunteers to form groups of three or four. (If it is a mixed gender class, try to make sure there are both men and women in each group.) In their groups, volunteers should read the topic **Breastfeeding** to themselves or as a group, whichever way they feel more comfortable.
 - a. After the group has completed reading, they should discuss the Reflection questions from the topic and be prepared for one of them to share their responses with the larger group during a plenary session.
 - b. Allow approximately 15 minutes for reading and discussion.
5. After 15 minutes, hold a plenary session and ask each group to present their responses to the Reflection questions. Monitor responses and put right any misunderstandings or misinformation.

Acceptance of breastfeeding in the community

1. Tell them that the first thing they must know about is whether breastfeeding in their community is usual. They will now explore among themselves what they know.

2. Ask the volunteers to go back to their small groups. Then ask them to respond, as far as they know, to the questions in the topic content titled Questions to pose to the community. Tell them that it is important for them to know whether most women give breast milk to their babies so that they can be sensitive with mothers and their families. It might, for example, be necessary, to tell them that some of their beliefs about breastfeeding need to be adjusted.
3. At the end of 15 minutes, bring the groups back to a plenary session, and ask each group in turn to give their responses to the first question, then the second and third, and so on. If the responses are different from one group to another, lead a discussion on why and what the common practice or belief might be.
4. Once you have a general agreement, note the areas where the common practice differs from the best advice. Tell them that they should think about ways to approach mothers and their families with sensitivity to give the best advice for a healthy and thriving newborn and infant. Tell them that as they learn more in this topic, they will see what the best advice about breastfeeding is.

Breastfeeding support – Benefits of breastfeeding poster

1. Ask the volunteers to find a partner and to look together in the volunteer guide topic at the section on breastfeeding support and the key messages to pass on to mothers and their families. With the knowledge of what is common in their community, ask them to think about and discuss how they would approach pregnant women and their families to encourage them to start breastfeeding as soon as the baby is born and to continue exclusively for the first six months. Give them five minutes to discuss with partners.
2. Hand out the **Benefits of breastfeeding** poster. Using the poster, ask partners to practise how they would explain the benefits of breastfeeding. Each partner should take a turn. They should think about when to start this advice and how to help pregnant women overcome any doubts they have about being able to breastfeed. Give them 10 minutes to practise.
3. Bring them back to a plenary session and lead a discussion on each of the key messages.
4. Tell them that a good way of helping new mothers in the community is to start a breastfeeding support group where the mothers can help and encourage each other to persist with breastfeeding exclusively for the first six months and where the volunteer can help with any problems and strengthen the key messages.

Benefits of breastfeeding reflection

Ask the following questions:

- a) How do you feel about bringing this message of breastfeeding benefits back to your community?
- b) Which concepts will be easy to explain?
- c) Which concepts will be difficult to explain?

Helping mothers learn to breastfeed and common problems with breastfeeding – **How to breastfeed; Common problems with breastfeeding**

1. Tell the volunteers that they can help new mothers learn to breastfeed even when they are finding it difficult at the beginning. They should be telling pregnant women that the baby should be put to the breast within one hour of birth, and it should naturally want to feed.
2. Hand out the posters **How to breastfeed** and **Common problems with breastfeeding**. Demonstrate the actions to place the new baby skin to skin in the right position to feed. Use the posters to point out the positions.
3. Ask the volunteers to tell you what the baby does when it is ready to breastfeed. They should respond that the baby will be licking and rooting (making sucking movements with the mouth). When starting to suck, mothers should allow the baby as long as it wants, and should be breastfeeding on demand 24 hours a day.
4. Explain that volunteers should stress to the mother that the baby should not be given anything else except breast milk for the first six months. No bottles or pacifiers (artificial teats) should be given.
5. Ask the volunteers to call out the kinds of problems that a new mother might have with breastfeeding. Prompt them if necessary.
6. Explain that some new mothers might have problems with breastfeeding because they are afraid that they do not have enough milk for the baby. The volunteer can reassure them that there will be plenty of milk and the more the baby feeds the more milk will be produced. Other mothers might have sore nipples or swollen breasts, but this can be quite normal at first and should get less of a problem when they breastfeed often.
7. Tell them that it is recommended by the World Health Organization that mothers living with HIV should breastfeed exclusively for the first six months, at the same time as taking their antiretroviral medicines, as this is the best chance for the baby not to get infected with the virus.

Wrap-up

1. Review the learning objectives on the flipchart and ask the volunteers if they feel they have achieved each one.
 2. Ask volunteers for any further questions or concerns. It may be necessary to revisit one or two issues to emphasize the importance of encouraging and supporting breastfeeding throughout the community, particularly in communities where it is not the normal practice.
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eCBHFA Facilitator guide

Module: Maternal and Newborn Child Health

Topic: Care during pregnancy

Overview

Pregnant women and those who care for them learn about health care during pregnancy, why pregnancy checks are so important, how to stay healthy during pregnancy, signs to watch for that may mean there is a problem during the pregnancy, and preparing to breastfeed immediately after birth.

Learning objectives

At the end of this topic, learners will be better able to:

- say why being checked is so important when a woman is pregnant and why she should make regular visits to a health worker during her pregnancy
- say what the health worker should do at each visit
- advise mothers and their families about getting ready for a safe birth
- give advice about what to eat to stay healthy during pregnancy
- prepare mothers to breastfeed once the baby is born
- advise who to put the mother in touch with in the community for care and support if problems arise
- teach care during pregnancy content to their community

Topic tools

- Go to the clinic if...poster
- Healthy foods for pregnant women poster
- Birth plan checklist poster
- Disease prevention checklist

Facilitator preparation

- Write the learning objectives for the topic on flipchart paper and post at the front of the room.
- Prepare copies of the volunteer guide and tools for each trainee.

Topic summary

Volunteers have an important role in helping a pregnant woman and her family to have a healthy pregnancy, safe birth and a healthy newborn baby. Advice includes telling a pregnant woman what happens when she gets checked by a health worker during her pregnancy and why it happens. The volunteer should also give information on the signs to watch out for during pregnancy that might mean there is a problem and how to make a birth plan. Advice also touches on eating well with good healthy foods during pregnancy, and how to prepare for breastfeeding and birth control once the baby is born.

Suggested outline of activities

Depending on the number of volunteers and amount of discussion, this topic will take approximately 2 hours, 25 minutes to complete.

Activity	Time
Introduction	30 min
Role of the volunteer	5 min
Pregnancy health checks and disease prevention checks	30 min
Reflection	10 min
Signs there may be a problem	20 min
Reflection	10 min
Giving advice on what to eat and drink	10 min
Planning the birth, preparing for breastfeeding and birth control advice	20 min
Wrap-up	10 min
Total time	2 hr 25 min

Introduction

1. Explain that volunteers have a key role in supporting a pregnant woman and her family during pregnancy. Explain that volunteers, both female and male, will learn about healthy pregnancy care including pregnancy health checks, disease prevention checks, how to recognize signs there may be a problem, advice about eating and drinking, planning the birth, preparing for breastfeeding and birth control advice.
2. Ask female volunteers who have children to share how they were cared for during their pregnancy and childbirth. Ask male volunteers to share their experiences about female family members who have recently gone through pregnancy and childbirth.
3. Ask all volunteers to tell you what the situation is in the community for pregnant women. You can prompt them for responses. For example, can women easily get health care during pregnancy? Do women typically give birth at home? Or in a health facility? Do they get the help of skilled birth attendants? Or traditional midwives? Is breastfeeding common?
4. Introduce the learning objectives on the flipchart. Tell them that they will be learning how to pass on what they learn today to people in the community.

5. Explain how they will meet each objective by outlining the agenda and activities.
6. Ask if they have any questions and if so, answer them. Tell them that they should continue to ask questions during the session, not wait until the end.
7. Distribute the volunteer guide topic: **Care during pregnancy**
 - Explain that volunteers will be taught how to use four tools during the session and that they will be expected to practise using these tools and demonstrate their skill in delivering the content.
 - Allow approximately five minutes for the volunteers to become familiar with the materials.
8. Instruct volunteers to form groups of three or four. In their groups, they should read the **Care during pregnancy topic** to themselves or as a group, whichever way they feel more comfortable.
 - After the group has completed reading, they should discuss the Reflection questions found in the topic and be prepared for one of them to share their responses with the larger group during a plenary session.
 - Allow approximately 40 minutes for reading and discussion.
9. After 25 minutes, hold a quick plenary session and ask groups to share their responses to the Reflection questions. Monitor responses and correct any misunderstandings or wrong information.

Role of the volunteer

1. Explain that volunteers can help mothers in their community to have a healthy pregnancy, safe birth and a newborn baby that grows and thrives.
2. They will do this by telling the mother that she should have regular check-ups with a health worker, at least four times during her pregnancy. These are called “antenatal” visits. They can also teach the pregnant woman and her family about the signs during pregnancy that there may be a problem, and what to do if any are seen. Volunteers can help with the preparation of a birth plan and give emotional support to the mother.
3. Tell them that this session will help them to be able to do all this, and that there are tools which will remind them of the important points when they come to talk to mothers and their families.

Pregnancy health checks and disease prevention checks — What to expect when visiting with a health worker poster; Disease prevention checklist

1. Ask the female volunteers who have children if they had antenatal health visits with a health worker when they were pregnant. If they did, ask if they can remember what the health worker checked.
2. Explain that volunteers must stress the importance of pregnancy health checks and disease prevention checks that will be conducted by a health worker. Emphasize the importance of at least four visits with a health care worker during the pregnancy.

3. Hand out **What to expect when visiting with a health worker and Disease prevention checklist** which describe what happens at these visits. Give volunteers a little time to look at the tools. Using the tools provide examples of the type of information that health workers will be gathering from pregnant women about their health as well as disease prevention.
4. Ask volunteers to confirm that they understand all of the checks and why they happen. Pick some at random and ask them to say what they would say to a pregnant woman. Correct any misunderstandings.
5. Ask volunteers to form pairs and give them 10 minutes where each take turns practising explaining to pregnant women what to expect during a visit with a health care worker.

Reflection

1. Once the pairs have completed their practice, bring the group together.
2. Ask the following questions:
 - a. How did their practice on explaining pregnancy health and disease prevention go?
 - b. What was easy to explain?
 - c. Which concepts were difficult to explain?
 - d. How do you feel about bringing this message back to your community?

Signs during pregnancy that there may be a problem — **Go to the clinic if... tool**

1. Ask volunteers if they know any of the signs during pregnancy that there may be a problem. Write these on a flipchart as they call them out. Refer to the topic content and add any that have not already been mentioned.
2. Hand out **Go to the clinic if...** tool and give volunteers a few minutes to get familiar with the poster. Show them how they can use the poster to advise pregnant women, fathers and the rest of the family about what problems to look for during pregnancy. Tell them that they will need to reassure pregnant women that most pregnancies will be normal and that these signs will not occur, but that if they do, the family needs to get medical help immediately.
3. Ask for two volunteers. One will role play a volunteer and the other will role play a pregnant mother. Ask the "volunteer" to explain to the pregnant mother the signs that there might be a problem during the pregnancy. Ask the rest of the volunteers to observe and to note where they might explain things differently or use different words.
4. At the end of the role play, ask what they liked about the role play. Then, ask what they might have said differently or added to what was told to the pregnant woman.

Reflection

Ask the following questions:

- a. What do you think will be easy to explain to a pregnant woman and her family about the danger signs during a pregnancy?
- b. Which concepts might be difficult to explain?
- c. How do you feel about bringing this message back to your community?

Giving advice on what to eat and drink — **Healthy foods for pregnant women tool**

1. Hand out the **Healthy foods for pregnant women tool** and give volunteers a couple of minutes to review the information.
2. Using the poster, explain to volunteers which foods a pregnant woman should eat so that she stays healthy during her pregnancy and has a healthy baby.
3. Review the signs of anaemia listed in the volunteer guide. Explain the importance of reminding pregnant women to take vitamins given to them by their health worker and watch for signs of anaemia.

Planning the birth — **Birth plan checklist tool**

1. Hand out the **Birth plan checklist** and give volunteers a couple of minutes to look over the poster.
2. Ask volunteers to get into groups of three or four. If possible, there should be both female and male volunteers in each group. Ask the groups to imagine that they are helping a pregnant woman and her family fill in the birth plan checklist.
3. Ask them to spend 15 minutes getting agreement among the group about possible responses and how they would encourage the family to give the birth plan a lot of thought. They should use the **Birth plan checklist** tool and start at the beginning of the path with the prompt “When is your baby due?” They should then follow the path and answer each question. Ask the group to write out their responses. Written responses will be posted and shared with the group.
4. At the end of the time, ask each group to share their responses.
5. Collect the responses and post at the front of the room for the other groups to see ideas and differences. (Keep these for use in the **Preparing for a safe birth** topic.)

Preparing for breastfeeding and birth control advice

1. Explain to volunteers that discussions about breastfeeding and birth control may also take place when talking about a birth plan.
2. Refer briefly to breastfeeding in the context of good nutrition for the newborn baby, but tell the volunteers that they will be approaching the subject in more detail during the **Breastfeeding** topic.
3. Refer briefly to spacing children at least two years apart so that they have the best chance to grow and thrive, and relate this to birth control, which will also be taught in more detail in the **Family planning** topic.

Wrap-up

1. Review the learning objectives on the flipchart and ask the volunteers if they feel they have achieved each one.
2. Ask for any further questions or concerns. Revisit any areas where doubts are raised.
3. Ask them if they feel confident to use the tools provided and advise a woman about healthy pregnancy and regular visits to a health worker during pregnancy.

References and resources

Essential Interventions, Commodities and Guidelines for RMNCH: A Global Review of the Key Interventions Related to RMNCH, WHO, Aga Khan University and PMNCH, 2011.

WHO recommendations on antenatal care for a positive pregnancy experience, November 2016. See: apps.who.int/iris/bitstream/10665/250796/1/9789241549912-eng.pdf?ua=1.

Strengthening the capacity of community health workers to deliver care for sexual, reproductive, maternal, newborn, child and adolescent health. See: apps.who.int/iris/bitstream/10665/174112/1/WHO_FWC_MCA_15.04_eng.pdf.

What matters to women during pregnancy: a different approach to antenatal care, WHO 2016.
See: www.who.int/reproductivehealth/topics/maternal_perinatal/anc/en/.

Healthy Child Uganda 2015: Village Health Team Maternal and Newborn and Child Health Training Manual, Mbarara, Uganda, Government of Canada, Department of Foreign Affairs, Trade and Development (DFATD).
See: www.healthynewbornnetwork.org/hnn-content/uploads/VHT-MNCH-Training-Manual_2.pdf.

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Module: Maternal and Newborn Child Health

Topic: Family planning

Overview

Birth control helps couples decide whether and when they wish to have children. Volunteers can play an important role in advising on the benefits of birth control and healthy spacing of pregnancies. If they know what people normally do in their community about birth control, it will help them to be sensitive with messages. They can advise about methods of contraception to help couples choose the best method for their purposes, and they can show how to use both male and female condoms.

Learning objectives

At the end of this topic, volunteers will be better able to:

- tell people about why birth control is good for the health of the woman and the family
- talk to people about healthy timing between pregnancies
- tell people about places in the community that give birth control advice
- list common methods of birth control
- show how to use a condom correctly
- teach family planning content as well as condom use to their community

Suggested outline of activities

Depending on the number of volunteers and amount of discussion, this topic will take approximately 2 hours, 30 minutes to complete.

Activity	Time
Introduction	25 min
Talking to the community	45 min
Reflection	10 min
Birth control methods	60 min
Wrap-up	10 min
Total time	2 hr 30 min

Tools

- Birth control methods chart
- Which method is best for me?
- Using male condoms
- Using female condoms
- Samples of methods, e.g. condoms, pills, IUDs, implants, injectables

Preparation

- Write out the learning objectives on a flipchart and post at the front of the room.
- Write discussion questions for Introduction on flipchart paper.
- Prepare copies of the volunteer guide topic and tools for each participant.
- Acquire samples of condoms and anatomical models (stick or banana) to demonstrate condom use. Acquire samples of other methods in use in your community, such as pills, IUDs, etc.

Introduction

1. Welcome participants to this topic. Explain that they are going to explore how best to talk about birth control or contraception with community members by telling them about why spacing births can improve the health of mothers and children. They are also going to be able to describe the various methods of contraception and give demonstrations of correct condom use.
2. Introduce the topic objectives and explain the agenda and activities that they will do to meet those objectives.
3. Distribute the topic: **Family planning** from the Volunteer guide.
 - a. Explain that volunteers will be taught how to use three tools during the session and that they will be expected to practise using these tools and demonstrate their skill in delivering the content.
 - b. Allow approximately two minutes for the volunteers to become familiar with the topic materials.
4. Instruct volunteers to form groups of three or four. (If it is a mixed gender class, try to make sure there are both men and women in each group.) In their groups, volunteers should read the **Family planning** topic to themselves or as a group, whichever way they feel more comfortable.
 - a. After the group has completed reading, they should discuss the Reflection questions found in the topic and be prepared for one of them to share this with the larger group during a plenary session.
 - b. Allow approximately 15 minutes for reading and discussion.
5. After 15 minutes, hold a quick plenary session and ask each group to share their responses to the Reflection questions. Monitor responses and correct any misunderstandings or misinformation.

Talking to the community

1. Explain that volunteers will discuss what people in their home community usually do about contraception. Ask them to work with a partner (mixed gender if you have both male and female participants). Ask them to discuss the following questions (post the questions on the flipchart at the front of the room). Allow 10 minutes for discussion.
 - Is contraception practised commonly in the community?
 - If so, what is the usual method?
 - Who normally takes responsibility for contraception – the woman or the man?
 - If contraception is not practised commonly, are there any taboos about the practice?
 - At what age do women usually start to be sexually active?
 - Are there very young mothers in the community, i.e. under 18 years? Or older mothers, over 35 years?
 - Are there large families in the community?
 - Are there places that people (men, women and adolescents) can go to get advice about birth control?
 - What are the main barriers to birth control?
2. At the end of 10 minutes, ask each pair to share their responses with the plenary session. Tell them that this is the situation in which they will be working and they should be thinking about the most sensitive way of approaching community members with their information.
3. Explain that birth control, also called family planning, allows couples and individuals to choose whether and when to have children and how many. This is one benefit, but more importantly, spacing pregnancies at least two years apart improves the health outcomes for both mothers and their children. Tell them that volunteers have an important role in breaking down barriers to birth control use and in advising on methods. They can also work together with community members to try and improve access to places where families can get advice on birth control.
4. Ask volunteers for ideas about what actions they can take in their home community to support family planning. Record these on a flipchart. Lead a plenary discussion on how some of these ideas might work and what kinds of community discussion and action they could put in place.

Reflection

Ask the following questions:

- a) How do you feel about talking to your home community about birth control? Do you feel comfortable talking with women and men about the topic?
- b) Which concepts will be easy to explain or discuss?
- c) Which concepts will be difficult to explain or discuss?

Birth control methods – **Birth control methods chart; Which method is best for me?; Using male condoms; Using female condoms**

1. Ask volunteers which methods they think are the most effective for birth control? Mark responses on flipchart paper. Then, ask what methods are the least effective?
2. Hand out the **Birth control methods chart**. Give volunteers about 5 minutes to review the content. Point out the most effective and least effective birth control methods. Note that not all methods may be available in your country or community. (If you have samples of the various methods, show to the volunteers.)
3. Hand out the **Which method is best for me?** poster. Ask volunteers to discuss with a partner how they might use this tool to help their community members make better decisions about family planning. After 7 to 10 minutes, ask volunteers to share their ideas in a plenary session.
4. Tell them that now they are going to practise showing how to use a male condom and then a female condom. Male condoms might be the most readily available contraceptive product in the community, and as well as protecting against pregnancy, they also protect both partners against sexually-transmitted infections, including HIV/AIDS.
5. Hand out the **Using male condoms** poster. Using your anatomical model (stick or banana) show how to use a male condom. State each step aloud as you demonstrate.
6. Give each volunteer a condom. Ask them to get into groups of three and give each group a stick or banana. Ask them to take turns playing one of these roles: volunteer, community member, observer. They should practise teaching each other how to use a condom correctly. Using the poster, observers should mark off each step as it is completed and then give feedback after each practice. (Alternate activity: cut and paste the steps of putting on a male condom.)
7. Hand out **Using Female condoms** poster. Show participants a female condom and go through the steps on the checklist for female condom use. Show how to squeeze the inner ring. Pass the condom around so they can feel the ring and how to squeeze it. (Additional activity: cut and paste the steps of putting on a female condom.)

Wrap-up

1. Revisit the learning objectives and ask volunteers if they feel they have achieved them. Ask them if they feel confident about approaching community members with the messages about birth control and healthy spacing of pregnancies.
 2. Ask them if they feel confident about giving information about birth control methods and showing community members the use of condoms.
 3. Ask for any questions or issues. If they are able, get participants to answer questions, and correct them if necessary.
 4. Thank them for their participation.
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eCBHFA Facilitator guide

Module: Maternal and Newborn Child Health

Topic: Female genital cutting

Overview

This topic addresses what female genital cutting is, why girls and women are cut and who does the cutting as well as the risks and problems associated with cutting and how we can discuss ways to reduce and eliminate harmful cutting practices.

Learning objectives

At the completion of this topic, volunteers should be able to:

- say what female genital cutting/mutilation is (referred to in this topic as female cutting)
- explain why female cutting happens
- explain how female cutting harms girls and women
- discuss ways to prevent female cutting in your community
- tell girls and women where they can go in their community for help and advice about female cutting
- teach the risks and problems associated with female genital cutting to their community

Suggested outline of activities

Depending on the number of volunteers and amount of discussion, this topic will take approximately 2 hours, 40 minutes to complete.

Activity	Time
Introduction	35 min
What is female genital cutting?	40 min
The health risks of cutting	20 min
Talking to the community	30 min
Reflection	10 min
Alternatives to cutting	25 min
Wrap-up	10 min
Total time	2 hr 50 min

Tools

- Reasons why female cutting still exists
- What religion says about female cutting
- Effects of female cutting on women's health
- Doer/non-doe analysis (Assessment module) referenced but not used in the module

Preparation

- Write the learning objectives for the topic on flipchart paper and post at the front of the room.
- Write discussion questions for What is genital cutting? on flipchart paper.
- Prepare copies of the volunteer guide topic and tools for each trainee.

Topic summary

Within the community, the volunteer can have an influence on the ending of female cutting. With knowledge about the harmful effects on women's health, both physical and mental, and with information about why cutting is practised and who supports it, volunteers can give strong messages about the practice. They can also talk with different groups in the community to tailor the message accordingly. They can work with groups of people in the community to discuss and put in place alternatives to cutting which would still retain an element of the cultural tradition.

Before we begin...

Female genital cutting or mutilation is an incredibly sensitive topic for many communities. To start the session, it is strongly suggested to recognize the sensitivity of this topic and ask permission of any elders who may be present in the training room. If no elders are present, it is still important to sensitize the group by ensuring that this topic is designed to start discussion and educate – not blame or judge. The female facilitator/volunteer should properly sensitize the leaders and elders of the community before beginning to discuss this topic.

Introduction

1. Welcome the volunteers to this session. Introduce the learning objectives and go through the agenda and activities they will be doing to meet those objectives.
2. Explain that the subject matter in this topic is very sensitive and is one that they have not touched on in the other topics. They will need to explore ways to approach community members carefully and with a well-thought-out plan of what to do and say. But daring to talk about it at all is the first step.
3. Distribute the topic **Female genital mutilation/cutting** from the Volunteer guide.
 - a. Explain that volunteers will be taught how to use three tools during the session and that they will be expected to practise using these tools and demonstrate their skill in delivering the content.
 - b. Allow approximately two minutes for the volunteers to become familiar with the topic materials.

4. Instruct volunteers to form groups of three or four. (If it is a mixed gender class, try to make sure there are both men and women in each group.) In their groups, volunteers should read the **Female genital mutilation/cutting** topic to themselves or as a group, whichever way they feel more comfortable.
 - a. After the group has completed reading, they should discuss the Reflection questions found in the topic and be prepared for one of them to share their responses with the larger group during a plenary session.
 - b. Allow approximately 25 minutes for reading and discussion.
5. After 25 minutes, hold a quick plenary session and ask each group to share their responses to the Reflection questions. Monitor responses and correct any misunderstandings or misinformation.

What is female genital cutting?

1. Review the topic content about female genital cutting. Explain that there are different types of cutting, some more severe than others.
2. Post the prepared flipchart paper with discussion questions at the front of the room. Explain that they will be discussing the tradition of female cutting in their own community. Review the discussion questions:
 - What kind of cutting is taking place? Why is it done?
 - Is cutting legal? If not, is it done in secret?
 - Who does the cutting?
 - At what age are girls or women cut?
 - What would happen if parents decided not to get their daughter cut?
3. Ask volunteers to form groups of four (mixed gender if you have both male and female participants) to discuss the questions considering their home communities. Give them 15 minutes.
4. At the end of 15 minutes, ask them to come to a plenary session and ask each small group to share their responses.

Why do people practice FGC? – Reasons why female cutting still exists poster

1. Hand out the **Reasons why female cutting still exists** poster.
2. Review the four reasons/beliefs about female cutting.
3. If there is agreement about the practice in the community, lead a discussion about why people in their community want the practice to continue. Take each statement from the five reasons and ask them which ones are believed in their community. Which are the most important reasons?
4. Tell participants that once they have a good knowledge of the reasons people continue to cut girls, they will be able to approach them with sensitivity. This is particularly true when cutting is a strong tradition and has been practised as part of the culture for a long time. Many people in the community will have strong opinions and it will not be easy to change their minds. Tell them that it may even be the case that older women who have been cut themselves wish to continue the practice as a matter of honour.

The health risks of cutting – **Effects of female cutting on women's health poster**

1. Hand out the **Effects of female cutting on women's health** poster. Review the content for immediate health problems. Point out that these are mostly physical, but that it is likely they will lead to mental problems too which are not so visible. Tell participants that cutting may even cause the death of the girl or woman.
2. Review content on long-term health problems, and point out that many of these problems affect not just the girl or woman, but the entire family. A woman may become infertile, suffer miscarriages, have difficulty with childbirth and have a less healthy baby. A woman may suffer severe depression, anxiety and low self-esteem.
3. Ask participants to brainstorm what the effects of some of the long-term problems might be on other members of the family, on a husband, mother, father, child, for example. Write their responses on chart paper as they call out. At this stage, do not comment.
4. Once you have a list of effects, look at each one and lead a discussion on how families are affected. You may find that you can go more deeply into some of the effects, to note for example, the fact that a family's income might suffer because of increased health service needs throughout the woman's life. Then, ask about the effect on the whole community. This discussion should give the volunteers some clear ideas about the best approach to community members to share their knowledge.

Talking to the community – **What religion says about female cutting poster**

1. Hand out **What religion says about female cutting** poster. Review the tool content.
2. In their small groups, ask volunteers to discuss how they might get a community group together and what they would say to them. Ask them to make a list of headings to cover the most important points to make. They should be guided by the section about the role of the volunteer. Give them 15 minutes to discuss.
3. Come back to a plenary session and ask groups to share their ideas. Make sure that they mention: immediate and long-term health problems, illegality of the practice, religion against the practice (religion not the same as tradition), effect on relationships, effect on families, effect on the community.
4. Tell them that they might try to bring small groups of community members together at first to raise greater awareness of the issue. They might also bring groups of girls together to educate them on why female cutting happens and on their right to decide for themselves what happens to their bodies. Cutting is a violation of their rights. The girls might like to form clubs for uncut girls where they support each other and lead efforts to end female cutting.
5. Tell participants that in many cultures, women who are uncut are denied a normal social life. They are less likely to be wanted in marriage and may suffer abuse from their neighbours. Volunteers might try to bring groups of women, both cut and uncut, together, to share their stories and become more familiar with each other's lives.

6. Explain that cutting will not stop unless the attitudes of men and boys can be changed. Volunteers can bring together small groups of men and boys to talk about protecting their womenfolk and about the harmful effects of cutting. Volunteers can lead a discussion about perceived purity, beauty or cleanliness and whether cutting is the way to guarantee these.
7. Explain that older women are often strongly in favour of cutting. They will probably have been cut themselves and consider it an honour. Volunteers might try to bring a group of grandmothers together to discuss their experiences, to find out if they have suffered health problems, whether physical or mental, and to explore why they think cutting should continue. Changing their minds will not be easy, but volunteers can give them facts and appeal to their sense of family and community.
8. Ask volunteers to consider using the **Doer/non-doer analysis** tool in the *Assessment* module to learn about the barriers to change and identify the best strategy to use to change beliefs in their home communities.

Reflection

Ask the following questions:

- a) How do you feel about talking to your home community about female genital cutting? Do you feel comfortable talking with women and men about the topic?
- b) Which concepts will be easy to explain or discuss?
- c) Which concepts will be difficult to explain or discuss?

Alternatives to cutting

1. Tell volunteers that in some countries, communities who want to protect their girls and women are finding different ways of celebrating girls becoming women. This might be special ceremonies or “parties” where the women receive gifts.
2. Ask them to brainstorm what they might suggest to communities as alternatives to cutting.
3. After you have several ideas, follow up on which ones seem to be most practical and lead a discussion on how they might work. The idea is to think of events or practices that might appeal to communities and take the place of cutting.

Wrap-up

1. Revisit the learning objectives and ask the volunteers if they feel they have achieved them.
2. Ask if they feel confident about approaching the subject of female cutting with community members. Ask if they have any specific concerns or issues with talking to community members.
3. Ask if they have any questions, and revisit parts of the topic if you feel that they need to go deeper into the subject.

References and resources

WHO, UNICEF, UNFPA Joint Statement 1997
apps.who.int/iris/handle/10665/41903

WHO FGM Fact Sheet February 2018.
www.who.int/news-room/fact-sheets/detail/female-genital-mutilation

Costello, S. Female genital mutilation/cutting: risk management and strategies for social workers and health care professionals. *Risk Management and Healthcare Policy*. 2015; 8: 225-233.

Ending Female Genital Mutilation/Cutting: Lessons from a Decade of Progress. Population Reference Bureau, 2013.
www.prb.org/progress-ending-fgm/

FGM frequently asked questions. UNFPA, January 2017.
www.unfpa.org/resources/female-genital-mutilation-fgm-frequently-asked-questions

Female Genital Mutilation/Cutting: A Global Concern UNICEF, New York, 2016.
data.unicef.org/resources/female-genital-mutilationcutting-global-concern/

Rigmore C. Berg and Vigdis Underland. The Obstetric Consequences of Female Genital Mutilation/Cutting: A Systematic Review and Meta-Analysis. Review Article Norwegian Knowledge Center for the Health Services, Oslo, Norway, June 2013.

Eliminating female genital mutilation: an interagency statement UNAIDS, UNDP, UNECA, UNESCO, UNFPA, UNHCHR, UNHCR, UNICEF, UNIFEM, WHO 2008.
www.un.org/womenwatch/daw/csw/csw52/statements_missions/Interagency_Statement_on_Eliminating_FGM.pdf

Toubia NF and Sharief EH. Female Genital Mutilation: have we made progress? *Int J Gynaecol Obstet*. 2003 Sep;82(3):251-61.

Yee, A. Talking it Out: The Effort to End Female Genital Mutilation in Ethiopia. *UNDARK*, 4-6-2017.

FGM: Integrating the prevention and the management of the health complications into the curricula of nursing and midwifery (WHO). See:
www.who.int/gender/other_health/teachersguide.pdf.

Amusan OA, Asekun-Olarinmoye EO. Knowledge, beliefs, and attitudes to female genital mutilation (FGM) in Shao Community of Kwara State, Nigeria. *International Quarterly of Community Health Education*, 2006–2007, 27:337–349.

eCBHFA Facilitator guide

Module: Maternal and Newborn Child Health

Topic: Menstrual hygiene

Overview

Approaching the sensitive and often taboo subject of menstruation (a girl or woman's monthly bleeding or period); giving the facts about menstruation to girls and women, boys and men; helping the community to provide safe and hygienic places for girls and women when they have their period.

Learning objectives

At the end of this topic, volunteers should be able to:

- explain the role of the volunteer to support menstrual hygiene
- describe the basic facts about menstruation
- explain why hygiene is so important when women have their period
- provide girls, women, boys and men with facts about good menstrual hygiene
- promote mutual understanding and respect between girls, women, boys and men about menstruation and menstrual hygiene
- support the community to find ways to improve menstrual hygiene
- identify where girls and women can go in the community to get help on menstrual hygiene issues
- teach menstrual hygiene content to their community

Suggested outline of activities

Depending on the number of volunteers and amount of discussion, this topic will take approximately 2 hours, 15 minutes to complete.

Activity	Time
Introduction	20 min
Key messages about menstruation	15 min
Reflection	10 min
Key messages about good menstrual hygiene practices and role play	60 min
Community support for menstrual hygiene	20 min
Wrap-up	10 min
Total time	2 hr 15 min

Tools

- Key messages about menstruation poster
- Facts and misunderstandings about menstrual hygiene poster
- Disposable pads posters
- Reusable pads posters

Preparation

- Write the learning objectives for the topic on a flipchart and place at the front of the room.
- Prepare copies of the volunteer guide and tools for each trainee.

Topic summary

Women having their monthly period (menstruation) is often a taboo subject, but it is a natural function that should be understood and respected by all members of the community, including men and boys. Volunteers can play an important role in giving the facts about menstruation and good menstrual hygiene, as well as getting support from the community to improve the area and provide safe, clean facilities for women who are having their period.

Introduction

1. Welcome volunteers to the session. Tell them that although menstruation is often a taboo subject, it is important to include all community members in the messages that the volunteers will pass on to the community, so that men and boys will understand and respect the function and the needs of the girls and women when they are having their period.
2. Introduce the learning objectives. Tell them that they should apply to the whole community, not just the female members.
3. Tell them that improving good menstrual hygiene in the community might mean working with community members to build safe and private latrines and washing facilities, action which community members as a whole can support.
4. Hand out the topic **Menstrual hygiene** from the volunteer guide.
 - Explain that volunteers will be taught how to use two tools during the session and that they will be expected to practise using these tools and demonstrate their skill in delivering the content.
 - Allow approximately two minutes for the volunteers to become familiar with the topic volunteer guide.
5. Instruct volunteers to form groups of three or four people. In their groups, volunteers should read the **Menstrual hygiene** topic to themselves or as a group, whichever way they feel more comfortable.
 - After the group has completed reading, they should discuss the Reflection questions found in the topic and be prepared for one of them to share their responses with the larger group during a plenary session.
 - Allow approximately 15 minutes for reading and discussion.
6. After 15 minutes, hold a plenary session and ask groups to share their responses to the Reflection questions. Monitor responses and correct any misunderstandings or misinformation.

Key messages about menstruation – Key messages about menstruation poster

1. Hand out the **Key messages about menstruation poster**.
2. Explain how this poster can be used to start a discussion with community members about important facts on menstruation. This tool can be used with any gender and age.
3. As you hold up the tool, explain each icon.
4. Be sure to check in frequently with participants and ask if anyone has any questions. Answer them the best that you can. If you do not know the answer, be honest and get back with an answer when you are able.
5. Remind volunteers that this information should be available to both girls and women and boys and men within their community.
6. Ask the volunteers to get into pairs. Pair men with women to ensure gender balance and to help make both genders more comfortable in delivering or at least, understanding, this sensitive material. Give them 10 minutes where they each take turns practising delivering the content.
7. Once the pairs have completed their practice, bring the group together.

Menstruation message reflection

Ask the following questions:

- How did the practice on the menstruation messaging go?
- What was easy to explain?
- Which messages were difficult to explain?
- How do you feel about bringing this message back to your community?

Key messages about good menstrual hygiene practices – Facts and misunderstandings about menstrual hygiene poster

1. Hand out the **Facts and misunderstandings about menstrual hygiene** poster.
2. Explain that this tool can be used to start a discussion with community members about menstrual hygiene practices.
3. Explain the tool is a game with 15 statements, 9 statements are correct and 6 are incorrect. Ask volunteers individually to cut sentence strips and then identify each statement as FACT or MISUNDERSTANDING. Remind volunteers that they should use the volunteer guide to help them determine which statements are correct and which are incorrect. Each volunteer should complete the tool.
4. When completed, ask a few volunteers to share their responses.
5. Take each of the incorrect statements in turn and review why they are incorrect.
6. Be sure to ask if anyone has any questions about the facts.

Role play on misunderstandings and facts

Ask people to get into groups of five people. Each group will receive a role play scenario and prepare to act it out for the wider group. The scenarios identify some misunderstandings people may have in the target community. Below are some sample scenarios, but the facilitator should modify based on misunderstandings present in the communities where the volunteers work and live.

They will work in their groups for ten minutes to prepare their role play scenarios. Then they will be invited to come up and perform the role play. Sample scenarios include:

1. Women in the community are discussing the need to douche or direct scented water into their vaginas to clean them out. The eCBHFA volunteer explains how women should not use a douche or anything inserted into the vagina to clean it.
2. A man is trying to convince his woman to have sex, saying that the semen will clean out her vagina. He tells her that her vagina smells badly and she needs some semen. The eCBHFA volunteer explains that semen has no such cleaning properties and will actually make the smell stronger.
3. Women are complaining about the high cost for sanitary products. One woman explains that she makes her own sanitary pads from sewing absorbent cloth which she washes and reuses. The other women are skeptical but the eCBHFA volunteer explains the frequency that the pads should be changed for best vaginal health.
4. Women are complaining that their vaginas smell and discuss the best way to clean. The eCBHFA volunteer gives guidance.

After EACH role play is performed, hold a brief plenary asking:

- What was the situation?
- Were there any misunderstandings? How did the volunteer handle this?
- Would you have done anything differently? If so, what?

Community support for menstrual hygiene

1. Explain to volunteers that they can teach all members of a community about menstruation and good hygiene practices. In order for the community to support menstrual hygiene, all community members need to be informed. Explain to volunteers that existing community groups, teachers and community leaders can assist in this area.
2. Ask the small groups to discuss the following: Thinking about your own community, answer the following questions:
 - Who in your community would you like to see involved in supporting menstrual hygiene?
 - What are some of the actions or activities you would like to see in your community to support menstrual hygiene.

Allow 10 minutes for small groups to discuss.

3. Bring the whole group together and using flipchart paper, list responses to questions.

4. Refer volunteers to the final section in the volunteer guide, and remind them that they can work with the Water, Sanitation and Hygiene (WASH) team at their local Red Cross Red Crescent branch to find ways of helping the community to have safe and clean water, latrines and education. Ask:
 - How can eCBHFA volunteers work with WASH teams to ensure menstrual hygiene is considered in planning latrine placement and maintenance?
 - How can eCBHFA volunteers work with WASH teams to ensure there is sufficient soap and clean water to support menstrual hygiene?
 - Why is WASH's input necessary on menstrual hygiene regarding latrines and access to clean water and soap?
5. Refer to the new eCBHFA WASH module for more information.

Wrap-up

1. Revisit the learning objectives on the flipchart and ask the trainees if they feel they have achieved each of them.
 2. Ask if they feel confident now to give support to menstrual hygiene for girls and women. Which topics might be difficult to discuss?
 3. Ask for any further questions and issues to be raised, and revisit parts of the topic if necessary.
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eCBHFA Facilitator guide

Module: Maternal and Newborn Child Health

Topic: Newborn and new mother care

Overview

Advice for a new mother and her family on the care of her newborn; care for the new mother; what to look out for that may mean there is a problem with either the mother or the baby after the birth; how to prevent infection; good healthy food; emotional support for the mother.

What you will learn

At the end of this topic, learners will be better able to:

- show the mother and her family how to care for both mother and baby after the birth
- describe “kangaroo mother care” and why it is good for baby and mother
- list the signs that may mean there is a problem with the mother after the birth and what to do if any of the signs are seen
- list the signs that may mean there is a problem with the newborn after birth and what to do if any of the signs are seen
- describe how to protect the newborn from infection
- advise mothers and their families about the importance of healthy food and rest
- advise about birth control after the birth
- teach how to help a mother and her newborn to be healthy after birth

Topic tools

- **Kangaroo mother care** poster
- **Signs that mother is having a problem after birth** poster
- **Ways to keep newborns safe** poster
- **Two handfuls of signs** poster

Suggested outline of activities

Depending on the number of volunteers and amount of discussion, this topic will take approximately 1 hour, 55 minutes to complete.

Activity	Time
Introduction	20 min
Checking mother after birth	10 min
Caring the newborn after birth	20 min
Signs there may be a problem	20 min
Reflection	10 min
Keeping baby safe after birth	10 min
Further advice	15 min
Wrap-up	10 min
Total time	1 hr 55 min

Preparation

- Write the learning objectives for the topic on a flipchart and place at the front of the room.
- Prepare copies of the volunteer guide and tools for each trainee.
- Make copies of the posters for this topic.

Topic summary

Volunteers can help a new mother survive and thrive after the birth by advising her and her family on caring for mother and newborn, why breastfeeding is so good, signs for both mother and newborn that there may be a problem, visits by or to a health visitor after the birth, immunizations and birth control for the future.

Introduction

1. Welcome the volunteers to the topic and introduce the learning objectives from the flipchart. Tell them that they will be learning how to pass on what they learn today to people in the community. Explain how they will meet each objective by outlining the agenda and activities. Explain that they have already visited some of the contents in earlier topics, but they will now do so in more detail.
2. Ask mothers among the group to share what they can remember about the first few days of their first baby's life. Were they given support and help? If so, what kind of support? Did the rest of the family have any role in looking after the new mother and the baby? How confident did they feel about looking after the baby? The idea is to get some feedback about why support at the time is important.

3. Distribute the **Newborn and new mother care topic** from the volunteer guide.
 - Explain that volunteers will be taught how to use three tools during the session and that they will be expected to practise using these tools and demonstrate their skill in delivering the contents.
 - Allow approximately two minutes for the volunteers to become familiar with the topic material.
4. Instruct volunteers to form groups of three or four. In their groups, they should read the **Newborn and new mother care topic** to themselves or as a group, whichever way they feel more comfortable.
 - After the group has completed reading, they should discuss the Reflection questions found in the topic and be prepared for one of them to share their responses with the larger group during a plenary session.
 - Allow approximately 15 minutes for reading and discussion.
5. After 15 minutes, hold a quick plenary session and ask each group to share their responses to the Reflection questions. Monitor responses and put right any misunderstandings or misinformation.

Checking the mother after she has given birth

1. Explain that a volunteer can help the mother immediately after the birth by checking that everything is normal. As usual, before touching the mother or baby they should wash their hands. Volunteers can then check over the mother. This can be done by rubbing the stomach just below the belly button. The belly should feel firm. There should be little bleeding. In the case of bleeding, or if the belly is not firm or if the mother requires stitches, call for medical help.
2. Encourage skin-to-skin contact with the baby and breastfeeding.
3. Explain that a volunteer should tell the new mother that a health worker should come within a day of the birth to check her and the baby and to talk about things like immunizations.

Caring for the newborn after birth - Kangaroo mother care poster

1. Ask the volunteers to get into groups of four. If you have a gender mix, then they should make sure there are both men and women in each group.
2. Ask volunteers to look over the section on **Caring for the newborn immediately after the birth** and discuss among themselves the most important points. They have 10 minutes. Using flipchart paper, they should write down a list of words and actions that would remind them what to do to support the mother and newborn after delivery. Examples would be "heat – keep warm", "dry body – clean cloth", "baby's head – hat or cloth", "kangaroo mother care – skin to skin contact".
3. At the end of 10 minutes, ask them to come back to the main group, and then each group in turn, introduce their words and actions. It is fine if they have the same or similar words and actions from group to group.

4. Hand out the **Kangaroo mother care poster**. Lead a discussion about kangaroo mother care, showing how it is similar to the way a mother kangaroo carries her baby in a pouch. Using the poster and/or a doll, demonstrate the positioning of the newborn, emphasizing that normally it would be skin to skin.
5. Ask for two volunteers to demonstrate on each other while the others observe and comment, if necessary.

Check for signs that there may be a problem with mother or baby

Signs that mother is having a problem after birth poster

Two handfuls of signs poster

1. Hand out **Signs that mother is having a problem after birth poster**. Using the poster, explain the symptoms a mother may have that may mean there is a problem. New mothers and family members should know the signs. If mother or family members see any of these signs, they should get medical help immediately.
2. Ask volunteers to get out **Two handfuls of signs poster**. Explain that the five times two danger signs for newborns are called handfuls as each finger can represent a sign. Teach volunteers the mnemonics Tiny Baby Cries Are Sweet for the first hand and So You Can Be Ever-ready for the other hand. For the first hand: Tiny = temperature, Baby = breathing, Cries = crying, Are = active, Sweet = Sucking. For the second hand: So = sucked in, You = yellow (jaundiced), Can = convulsions, Be = Belly button, Ever-ready = Eyes.
3. Ask trainees to get into groups of three to role play telling a mother about the signs that there may be a problem. They will in turn play a volunteer, a new mother or member of the family and an observer. The volunteer should be expressive, using their fingers and acting out the danger signs, trying to make them memorable. For example, for fever, they can mop a brow, fan themselves, etc. After each role play, the mother and the observer should comment about how easy it was to understand what the danger signs are.

Checking for signs there may be a problem reflection

1. Ask the following questions:
 - Which concepts will be easy to explain?
 - Which concepts will be difficult to explain?
 - How do you feel about bringing this message of signs of a problem with mother or baby back to your community?

Keeping baby safe soon after birth - Ways to keep newborns safe poster

1. Hand out the **Ways to keep newborns safe poster**. Using the poster, review its content with volunteers.
2. If you are operating in an area where malaria is common, mothers should be told

that they and the baby must sleep under an insecticide-treated net every night.

3. Explain that volunteers should tell mothers that a new baby should see a health worker. The health worker will discuss newborn eye care, Vitamin K, weighing the baby to make sure it is not underweight and immunizations.
4. Explain that knowing how to protect the newborn from infection is an important part of what the volunteer advises. The cord attached to the baby's belly button (the umbilical cord) should be observed every day for any sign of infection and medical help sought if the signs are seen.
5. Remind volunteers as discussed in the Breastfeeding topic, that breastfeeding exclusively should be encouraged as that helps protect the baby from infection.

Further advice

1. Ask the trainees to remember back to the Care during pregnancy topic in this module, and the advice they should give about healthy food. After giving birth, a mother needs a greater amount and variety of foods, similar to the kinds of foods recommended before the birth.
2. In small groups of three or four, ask them to discuss what mothers in their community usually eat and if there are any food taboos. Are healthy foods easily available in the community? If there are food taboos, how would they approach mothers to advise about what they should be eating?
3. After a brief discussion, bring the groups back to a plenary session and lead a discussion on local foods and how to persuade mothers and families to eat nutritionally good food.
4. Remind trainees that they will also be advising that the mother should get plenty of rest, and this means the family supporting her to do so.
5. Explain that new mothers often need good emotional support. This can be provided by the volunteer, but the family should also be made aware of the need.
6. Advising about birth control and the good health benefits of spacing births can also be part of the role of the volunteer (see the Family planning topic for more detail).

Wrap-up

1. Revisit the learning objectives on the flipchart and ask the trainees if they feel they have achieved each of them.
2. Ask if they feel confident now to give support to both new mother and baby after the birth.
3. Ask for any further questions and issues to be raised, and revisit parts of the topic if necessary.

References and resources

WHO (2015), Counselling for Maternal and Newborn Health Care: A handbook for building skills. See: www.who.int/maternal_child_adolescent/documents/9789241547628/en/.

WHO recommendations on postnatal care for the mother and newborn (2013). See: apps.who.int/iris/bitstream/10665/97603/1/9789241506649_eng.pdf?ua=1

D19-25 immediate postpartum care

D26-E10 postpartum care

Global Health Media has produced a series of free training films including a Small Baby Series and Breastfeeding Practices. Films in development to be available soon include Helping Babies Breathe and Complications in Childbirth. See: www.globalhealthmedia.org.

Maternal and Child Survival Program, USAID/WHO. See: www.mcsprogram.org.

EENC (Early Essential Newborn Care)

See: iris.wpro.who.int/iris/bitstream/10665/97603/1/9789241506649_eng.pdf?ua=1.

The Healthy Newborn Network: See: www.healthynewbornnetwork.org/#filter=. resource.

WHO recommendations on Newborn Health: See: www.healthynewbornnetwork.org/hnn-content/uploads/WHO-MCA-17.07-eng.pdf.

LSHTM Informed decisions for actions in maternal and newborn health: See: www.healthynewbornnetwork.org/hnn-content/uploads/IDEAS_LSHTM_Report-2010-2017.pdf.

UNICEF, Improving maternal and newborn survival: See: www.healthynewbornnetwork.org/hnn-content/uploads/Examples-from-the-field_Improving-maternal-and-newborn-survival.pdf.

See Zambia and Ethiopia for CWH examples

Helping babies breathe training materials:

See: www.aap.org/en-us/advocacy-and-policy/aap-health-initiatives/helping-babies-survive/Pages/Course-Materials.aspx.

Kangaroo Mother Care: See: www.mchip.net/sites/default/files/mchipfiles/MCHIP%20KMC%20Guide.pdf.

KMC to reduce morbidity and mortality in LBW infants: See: www.cochrane.org/CD002771/NEONATAL_kangaroo-mother-care-reduce-mortality-and-morbidity-low-birthweight-infants

MJ Sankar, et al. When do newborns die? A systematic review of timing of overall and cause-specific neonatal deaths in developing countries: Journal of Perinatology. 2016 May; 36(Suppl 1): S1-S11.

eCBHFA Facilitator guide

Module: Maternal and Newborn Child Health

Topic: Post abortion care

Overview

What abortion is; giving help to a woman who has had an abortion, whether sudden or planned; what to expect; what are the signs that there may be a problem; caring for herself; helping with her feelings by being understanding and not judging.

Learning objectives

At the end of this topic, learners will be better able to:

- support a woman who has had either a sudden or planned abortion
- list the signs in a woman after an abortion that may mean there is a problem
- advise the woman about caring for herself and what to expect after an abortion
- tell the woman where to get help in the community if she has any problems or sees any of the signs that may mean there is a problem
- teach care after an abortion content to their community

Tools

- Post-abortion care poster
- Psychological first aid topic

Suggested outline of activities

Depending on the number of volunteers and amount of discussion, this topic will take approximately 1 hour, 45 minutes to complete.

Activity	Time
Introduction	15 min
Psychological first aid after abortion	40 min
Caring for herself after abortion	40 min
Wrap-up	10 min
Total time	1 hr 45 min

Preparation

- Write the learning objectives for the topic on a flipchart paper and post at the front of the room.
- Prepare copies of the volunteer guide and topic tool for trainees.

Topic summary

There are two types of abortion. An abortion that is unexpected and sudden is called a miscarriage. An abortion that is planned is called an induced abortion. Volunteers can help a woman to recover physically and emotionally after an abortion by giving emotional support, advising on expected changes to the body, on caring for herself and watching out for signs that there may be a problem after an abortion.

Introduction

1. Welcome the volunteers to the topic. Introduce the topic objectives.
2. Tell the volunteers that this topic is concerned with both those women who have a miscarriage, i.e. a sudden abortion, and those who have a planned (induced) abortion for whatever reason.
3. Ask the group if induced abortion is legal in their country, and if so is it practised in their community. If not, what do women do when they wish to end a pregnancy?
4. Tell them that whether the pregnancy was wanted or unwanted, the woman is likely to need extra emotional support as she may experience grief, guilt and feelings of loss, as well as experiencing changes within her body. Their role is to support the woman and her family, to advise on what to expect and any danger signs to look out for.
5. Distribute the **Post-abortion care** topic from the Volunteer Guide
 - a. Explain that volunteers will be taught how to use one tool during the session and that they will be expected to practice using the tool and demonstrate their skill in delivering the content.
 - b. Allow approximately two minutes for the volunteers to become familiar with the topic material.
6. Instruct volunteers to form groups of three or four. In their groups, volunteers should read the **Post-abortion care** topic to themselves or as a group, whichever way they feel more comfortable.
 - a. After the group has completed reading, they should discuss the Reflection questions found in the topic and be prepared for one of them to share their responses with the larger group during a plenary session.
 - b. Allow approximately 10 minutes for reading and discussion.
7. After 10 minutes, hold a plenary and ask each group to share their responses to the Reflection questions. Monitor responses and correct any misunderstandings or misinformation.

Psychological first aid (PFA) practice – PFA topic

1. Ask the volunteers to get into small groups of three people.
2. Explain that each group will engage in a small role play to provide PFA to one or two persons. There will be three rounds of PFA.
 - First round: one person plays a woman who just suffered a miscarriage. The other two people in the group are eCBHFA volunteers who provide PFA to this upset woman.
 - a. Second round: one person plays a woman who took pills to abort her pregnancy. Abortion is illegal in the country. One person plays the woman's partner. The third person plays the eCBHFA volunteer who tries to comfort the couple.
 - b. Third round: one person plays a woman who had an abortion after she was raped. The other two people play eCBHFA volunteers who provide PFA and possibly a referral.
3. After all three rounds, call the group together for a plenary session. Discuss:
 - What was difficult in these PFA sessions?
 - If you personally are not an advocate of abortion, how did you stay neutral and care for the person in front of you, rather than focusing on the act of abortion? Which Fundamental Principle does this represent?
 - What surprised you when you were in these different scenarios?

Caring for herself after abortion – Post-abortion care poster; When should you visit the health worker poster

1. Hand out the **Post-abortion care poster**. Ask the trainees to find a partner, and, using the poster, discuss between themselves how they would advise a woman who has recently had a miscarriage about the changes to expect in her body and how she should look after herself.
2. Ask them to think particularly about how they would give emotional support while at the same time giving practical advice on healthy food, rest, avoiding sexual relations for a few days and birth control (covered in more detail in the **Family planning** topic).
3. After a short time, ask the whole group to call out some ideas about being supportive at this time.
4. Hand out **When should you visit the health worker?** poster and ask them about the signs that there may be a problem after an abortion. Using the poster as a guide, ask them what they would advise the woman to do if she experiences any of the signs.
5. Explore with the whole group where a woman in this situation should go within the community for help or advice. Use a flipchart to list different kinds of help and advice that may be available, e.g. health facility, traditional midwife or healer.

Wrap-up

1. Explain that birth control should be a part of the advice that they give a woman after an abortion, since for her health, she should wait at least six months before the next pregnancy.
 2. Ask if they feel confident about approaching the subject of care after an abortion with community members. Ask if they have any specific concerns or issues with talking to community members. Which concepts might be difficult to explain?
 3. Revisit the learning objectives on the flipchart and ask the group if they feel that have achieved them all. Ask if there are any questions or issues. If there are, ask the other members of the group to answer if they are able.
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References and resources

WHO pregnancy, childbirth, postpartum, and newborn care: a guide for essential practice (2015)

See: apps.who.int/iris/bitstream/10665/249580/1/9789241549356-eng.pdf?ua=1.
B19-21 PAC

WHO Safe Abortion: technical and policy guidance for health systems

See: apps.who.int/iris/bitstream/handle/10665/70914/9789241548434_eng.pdf?sequence=1.
Chapter 2.3

WHO Health workers roles in providing safe abortion care and post abortion contraception

See: apps.who.int/iris/bitstream/10665/181041/1/9789241549264_eng.pdf

National Abortion Federation – Clinical policy guidelines for abortion care

See: 5aa1b2xfmfh2e2mk03kk8rsx-wpengine.netdna-ssl.com/wp-content/uploads/2017-CPGs-for-Abortion-Care.pdf.

PAC Consortium – See: pac-consortium.org/downloads/Essential%20Elements%20of%20Postabortion%20Care-Service%20Delivery%20Barriers%20and%20Resources%2005-06-2014.pdf

Mailman School of Public Health - PAC Clinical training for reproductive health in emergencies. See: www.mailman.columbia.edu/sites/default/files/pdf/raise_pac_trainer_guide_bw.pdf.

Ipas Woman-centered, comprehensive abortion care trainer's manual. See: iawg.net/wp-content/uploads/2017/08/1.-Ipas-WCCACTRNE14.pdf.

Post Abortion Care: A Tool for Client Counseling and Community Education and Unsafe Abortion, Family Care International 2005

eCBHFA Facilitator guide

Module: Maternal and Newborn Child Health

Topic: Preparing for safe birth

Overview

Helping pregnant women and their families to plan for the baby to be born at a clinic or hospital; advising them on what to do when labour begins; showing the family how to support the woman during labour, birth and afterwards; signs in the newborn that there may be a problem.

Learning objectives

At the end of this topic, learners will be better able to:

- advise a pregnant woman and her family about getting ready for a safe birth
- list the early signs of labour
- tell the family how to support a woman during labour, delivery and immediately after the birth
- describe signs that there may be a problem with the newborn
- know who the mother should contact in the community for care and support if there are problems
- teach the contents of the safe birth topic in their community

Suggested outline of activities

Depending on the number of volunteers and amount of discussion, this topic will take approximately 1 hour, 35 minutes to complete.

Activity	Time
Introduction	25 min
When labour begins	10 min
Preparing a birth plan practice	15 min
Help during labour, delivery and immediately afterwards	20 min
Reflection	10 min
Signs in newborns that there may be a problem	5 min
Wrap-up	10 min
Total time	1 hr 35 min

Topic tools

- Birth plan checklist poster
- Baby is coming! Early signs of labour poster
- Two handfuls of signs poster

Preparation

- Write the learning objectives for the topic on flipchart paper and post at the front of the room.
- Prepare copies of the volunteer guide topic and tools for each trainee. They should have already received **Go to the clinic if...** tool and **Birth plan checklist** tool. Be prepared to give out more to those trainees who do not have them to hand.

Topic summary

The volunteer has an important role in advising a woman and her family during pregnancy so as to prepare for a safe birth. As well as helping with the preparation of the birth plan, discussed in the **Care during pregnancy** topic, the volunteer should remind the family about signs to watch out for that there might be a problem, and what the early signs of labour are. The volunteer should also advise about how the family can support the woman during labour, the birth and immediately after the birth and be able to advise on signs in a newborn that there might be problems.

Introduction

1. Welcome trainees to this topic. Introduce the topic objectives and explain the agenda and activities that they will do to meet those objectives.
2. Using the birth plans they prepared in their small groups in the **Care during pregnancy** topic, remind them of the kind of plans that pregnant women and their families need to make to prepare for a safe birth. Tell them that they can support women and their families all the way through the process, from early pregnancy to beyond the birth.
3. Distribute the topic **Preparing for a safe birth** from the volunteer guide
 - a. Explain that volunteers will be taught how to use two tools during the session and that they will be expected to practise using these tools and demonstrate their skill in delivering the content of the tools.
 - b. Allow approximately two minutes for the volunteers to become familiar with the topic materials.
4. Instruct volunteers to form groups of three or four. In their groups, volunteers should read the **Preparing for a safe birth** topic to themselves or as a group, whichever way they feel more comfortable.
 - a. After the group has completed reading, they should discuss the Reflection questions found in the topic and be prepared for one of them to share their responses with the larger group during a plenary session.
 - b. Allow approximately 15 minutes for reading and discussion.
5. After 15 minutes, hold a quick plenary session and ask each group to share their responses to the Reflection questions. Monitor responses and put right any misunderstandings or incorrect information.

Preparing a birth plan practice – **Birth plan checklist**

1. Remind the volunteers of the exercise they did on the birth plan in the **Care during pregnancy** topic.
2. Now ask them to get into groups of three or four (they should try to have different people in their group from the original exercise). Give each group one of the responses that was posted on the wall and ask them to discuss each of the answers, following the pathway in the **Birth plan checklist**. They should be asking: Is this answer one that we can support? If not, how would we try to advise the pregnant woman and her family? For example, if the plan is to give birth at home, what could they say to try to persuade the family that the woman would be safer in a hospital or clinic?
3. At the end of 10 minutes, ask the groups to come back to a plenary session and lead a discussion on what would be recommended responses to each of the questions on the **Birth plan checklist**. Remind them that at each visit to the pregnant woman and her family, the birth plan should be checked over to make sure nothing has changed.

When labour begins – **Baby is coming! Early signs of labour poster**

1. Ask volunteers if they know some of the signs of beginning labour. Ask those female volunteers who have children if they can remember what labour was like. Write these responses on flipchart paper.
2. Hand out the **Baby is coming! Early signs of labour** poster. Using the poster, explain the early signs of labour from the topic content and what the woman should do to try and relax between contractions. As they are discussed, add signs of labour that are missing to the flipchart paper.

Help during labour, delivery and immediately afterwards

1. Review the topic content about what the family can do during labour to support a woman.
2. Explain what to do immediately after delivery with the newborn. Be brief and tell them that they will have more detail in the **Newborn and new mother care** topic. Refer to breastfeeding briefly, but say that it will be covered in greater detail in the **Breastfeeding** topic
3. Ask the volunteers to get into groups of three to role play a woman in labour, the volunteer supporting her and a member of the family. They should go through the advice on giving support, and act out giving her support while in labour, before the delivery itself. The volunteer should be advising the family to give fluids to the woman, help her to change position regularly, massage her back and shoulders and be positive. The volunteer can teach the woman how to breathe through her nose with a long breath and a “shhh” sound.

Reflection

1. Once the groups have completed their practice, bring the whole group together.
2. Ask the following questions:
 - a) How did their practice on advising a woman in labour go?
 - b) What was easy to explain?
 - c) Which concepts were difficult to explain?
 - d) How do you feel about bringing this message back to your community?

Signs in newborns that there may be a problem – Two handfuls of signs poster

Give out **Two handfuls of signs poster** showing signs for newborns that there may be a problem. Using your own hand and fingers, briefly explain the ten danger signs. Tell the trainees that they will be returning to these signs in more detail in the **Newborn and new mother care** topic and will then practise how to show new mothers and family members what to look for in the baby.

Wrap-up

1. Refer to the learning objectives for the topic on the flipchart. Ask volunteers if they feel confident about advising pregnant woman in their community about safe birth.
2. Ask if they have any questions, and if so, ask if there are any trainees who can answer them. Put right any misunderstandings or incorrect information.

References and resources

PATH basic delivery kit guide. See: www.path.org/publications/files/MCHN_BDKG.pdf

WHO safe childbirth checklist implementation guide. See: apps.who.int/iris/bitstream/10665/199177/1/9789241549455_eng.pdf?ua=1).

UNFPA Monitoring emergency obstetric care: A Handbook. See: www.unfpa.org/sites/default/files/pub-pdf/obstetric_monitoring.pdf.